

MAGNETIC RESONANCE IMAGING REPORT

REPORT INFORMATION

Report Title: Rohan Iyer Brain MRI Report
Document Type: IMAGING
Report Date: 25-Jul-2024
Accession No.: MRI-2024-0725-1148
Modality: MRI
Body Region: Brain

PATIENT INFORMATION

Patient Name: Rohan Iyer
Patient Identifier: NEW-PATIENT-001 (External Synthetic ID)
Date of Birth: 17-Nov-1997 (Age: 26)
Gender: Male
Blood Group: A+

CLINICAL INFORMATION

Ordering Physician: Dr. Meenakshi Subramaniam, DM Neurology
Clinical Indication: Recurrent headaches of approximately four months duration with migrainous features. Evaluation to exclude secondary intracranial pathology.
Relevant History: No prior neuroimaging. Neurological examination unremarkable per referring clinician.

TECHNIQUE

MRI of the brain was performed on a 3.0 Tesla scanner (Siemens MAGNETOM Vida) using the following sequences:

- Sagittal T1-weighted
- Axial T1-weighted (pre- and post-gadolinium)
- Axial T2-weighted
- Axial FLAIR (Fluid-Attenuated Inversion Recovery)
- Axial DWI (Diffusion-Weighted Imaging) with ADC map
- Axial SWI (Susceptibility-Weighted Imaging)
- Coronal T2-weighted
- Post-contrast axial and coronal T1-weighted

Intravenous gadolinium-based contrast agent (Gadovist 0.1 mmol/kg) was administered. No adverse reaction to contrast was observed.

FINDINGS

CEREBRAL HEMISPHERES:

No focal parenchymal signal abnormality identified. Grey-white matter differentiation is well preserved. No areas of restricted diffusion on DWI to suggest acute infarction. No evidence of haemorrhagic transformation.

POSTERIOR FOSSA:

Cerebellum, brainstem, and fourth ventricle appear normal. No mass or signal abnormality.

VENTRICULAR SYSTEM:

Lateral ventricles are symmetric and normal in size and configuration. Third and fourth ventricles are normal. No hydrocephalus.

MIDLINE STRUCTURES:

No midline shift. Septum pellucidum is midline. Corpus callosum is normal in morphology and signal.

EXTRA-AXIAL SPACES:

No extra-axial fluid collection. Sulcal spaces are age-appropriate. No subdural or epidural haemorrhage.

SELLA / PITUITARY:

Pituitary gland is normal in size and morphology. No sellar or suprasellar mass. Optic chiasm is unremarkable.

ORBITS:

Globes and extraocular muscles appear normal bilaterally. No retrobulbar mass or abnormal enhancement.

VASCULAR STRUCTURES:

Major intracranial vessels demonstrate normal flow voids. No evidence of aneurysm, arteriovenous malformation, or venous sinus thrombosis on SWI and post-contrast sequences.

POST-CONTRAST:

No abnormal parenchymal or meningeal enhancement following gadolinium administration.

CALVARIUM AND SCALP:

Normal. No osseous lesion.

INCIDENTAL FINDINGS:

Mild mucosal thickening in the right maxillary sinus (likely incidental inflammatory change). No other incidental findings of clinical significance.

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IMPRESSION

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1. No acute intracranial abnormality identified.
2. No acute infarction, intracranial haemorrhage, or significant mass effect.
3. No space-occupying lesion. No abnormal enhancement.
4. Normal MRI of the brain.
5. Incidental mild right maxillary sinus mucosal thickening – likely clinically insignificant.

RECOMMENDATION:

Clinical correlation recommended. The imaging findings do not explain the patient's headache symptoms. Further management at the discretion of the referring clinician.

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RADIOLOGIST SIGNATURE

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Reported by:

Dr. Ashwin Kulkarni, MD Radiology
Senior Consultant Radiologist
Sanjeevan Neurosciences Clinic – Diagnostic Imaging Centre, Bengaluru

Date: 25-Jul-2024

Verified by:

Dr. Sunita Rao, MD DNB Radiology
Head, Department of Diagnostic Imaging

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DISCLAIMER: This report is based on the imaging findings at the time of examination and should be interpreted in conjunction with the clinical history and findings of the treating physician.

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Sanjeevan Neurosciences Clinic – Diagnostic Imaging Centre

Page 1 of 1

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